Carboplatin AUC 1.5 with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Seven weekly carboplatin AUC 1.5 visits after induction

Active medicines: Carboplatin

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone	8 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	PRN Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Carboplatin AUC 1.5 intravenous (IV) every 7 days for 7 cycles, only while radiotherapy is being delivered. Discontinue carboplatin if radiotherapy is truncated.
- The treatment paragraph describes a start 3–8 weeks (Days 22–56) after the START of cycle 3 of induction; the indication paragraph says 3–8 weeks after COMPLETION of induction. The multidisciplinary team (MDT) must resolve the reference point and set the actual radiotherapy and chemotherapy dates. Confirm the induction route and adequate recovery.
- NCCP gives no routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) or additional supportive-care prescription for this weekly phase. Do not carry over induction G-CSF, docetaxel premedication or cisplatin hydration automatically.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.
- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00332 v4 · pNCCP 00332: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00332 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required